



Patient:

Med. Record. No.:

Date of Birth:

Submitted by:

Report also to:

Age:

Sex: F

Pathology #:

Date of Procedure:

Date Received:

Pathology Report

DIAGNOSIS:

SPECIMEN

Urinary bladder, cystectomy

TUMOR

Histologic type: Urothelial (transitional cell) carcinoma

Histologic grade: High grade

Tumor size (greatest dimension): 2.2cm

Tumor configuration: Papillary

Tumor location: right and left walls

Extent of invasion: Invasion into muscularis propria (detrusor muscle)
(inner half)

Direct invasion of other organs/structures: None identified

Venous/lymphatic invasion: Absent

Associated epithelial lesions: None identified

MARGINS

All margins negative for malignancy

Nearest margin: Perivesical soft tissue margin

Distance to nearest margin: 5cm

Comment: Right and left ureter margins (specimens A and B) are benign

LYMPH NODES

All sampled lymph nodes are benign

Total number of nodes evaluated: 38

Comment:

Lymph node dissections include:

C - right obturator (5 benign nodes)

D - right paracaval (1 benign node)

E - right common iliac (adipose only, no lymphoid tissue)

F - left external iliac (8 benign nodes)

G - left obturator (7 benign nodes)

H - right external iliac (3 benign nodes)

I - left common iliac (3 benign nodes)

J - left para-aortic (2 benign nodes)

K - perivesical (5 benign nodes)

ICD-O-3
Carcinoma, urothelial
papillary 8130/3
Site ^{C67.9} Bladder NOS
^{C67.9} Bladder wall NOS
J11/29/14

Patient: [REDACTED]

MRN: [REDACTED]

L - sacral (4 benign nodes)

STAGING (AJCC)

Primary Tumor: pT2a

Lymph Nodes: pN0

Distant Metastasis: Cannot be assessed

CLINICAL INFORMATION:

Bladder cancer

SPECIMEN(S) RECEIVED:

- A. Right distal ureter
- B. Left distal ureter
- C. Right obturator
- D. Right paracaval
- E. Right common iliac
- F. Left external iliac
- G. Left obturator
- H. Right external iliac lymph nodes
- I. Left common iliac
- J. Left para-aortic
- K. Bladder
- L. Sacral nodes

GROSS DESCRIPTION:

A. Received fresh is a segment of ureter measuring 8 mm in diameter x 5 mm in thickness. Entirely submitted for frozen section. The frozen section remnant is submitted for permanent sections in A1.

FROZEN SECTION DIAGNOSIS: Benign ureter margin. Reported to [REDACTED] intraoperatively at [REDACTED] by [REDACTED]

B. Received fresh is a segment of ureter measuring 8 mm in diameter x 5 mm in length. Entirely submitted for frozen section. The frozen section remnant is submitted for permanent sections in B1.

FROZEN SECTION DIAGNOSIS: Benign ureter margin. Reported to [REDACTED] intraoperatively at [REDACTED] by [REDACTED]

C. Received in formalin is an 8.0 x 4.2 x 2.0 cm irregular piece of adipose tissue containing multiple partially fat-replaced lymph nodes ranging from several millimeters up to 4.8 x 2.0 x 1.2 cm. Sections: C1 - multiple small nodes, intact; C2 - one lymph node; C3-C8 - one lymph node.

D. Received in formalin is a 4.5 x 2.0 x 0.9 cm piece of adipose tissue containing a single 3.0 x 1.0 x 0.6 cm lymph node. The node is sectioned and entirely submitted in D1 and D2.

E. Received in formalin is a 2.5 x 2.5 x 0.9 cm aggregate of adipose tissue containing several prominent vessels. No distinct lymph nodes are identified. Entirely submitted in E1 and E2.

F. Received in formalin is a 4.5 x 4.0 x 1.5 cm aggregate of adipose tissue containing multiple lymph nodes ranging from several millimeters up to 2.2 x 1.4 x 0.8 cm. Sections: F1 - two smallest nodes, intact; F2 - one lymph node; F3 - one lymph node; F4 - one lymph node; F5 - one lymph node; F6 - one lymph node; F7 - one lymph node.

G. Received in formalin is a 5.0 x 3.5 x 1.5 cm aggregate of adipose tissue containing multiple lymph nodes ranging from several millimeters up to 4.3 x 1.5 x 0.7 cm. Sections: G1, G2 - smallest nodes, intact; G3-G5 - largest lymph node.

H. Received in formalin is a 3.7 x 3.0 x 0.9 cm aggregate of adipose tissue containing several small lymph nodes measuring up to 0.7 x 0.6 x 0.4 cm. All nodular tissue is submitted in H1.

I. Received in formalin is a 3.5 x 3.3 x 1.0 cm aggregate of adipose tissue containing several lymph nodes ranging from 0.7 x 0.5 x 0.2 cm up to 1.3 x 1.2 x 0.8 cm. Sections: I1 - smallest nodes, intact; I2 - one lymph node, bisected.

Patient: [REDACTED]

MRN: [REDACTED]

J. Received in formalin is a 2.5 x 2.2 x 0.5 cm aggregate of adipose tissue containing two probable lymph nodes measuring 0.3 x 0.2 x 0.2 cm and 0.9 x 0.8 x 0.3 cm. Submitted intact in J1.

K. Received fresh is a radical cystectomy specimen with 10.0 x 6.5 x 5.2 cm bladder surrounded by perivesical adipose tissue ranging from 0.7 to 3.2 cm in thickness. The urethra measures 5.0 cm in length x 1.7 cm in diameter. Overlying the posterior surface of the urethra is a strip of vaginal mucosa measuring 8.0 cm in length x 3.5 cm in width. The opened bladder displays a 4.0 x 2.5 cm ulcer on the right side. Multifocal residual papillary tumor is noted adjacent to the ulcer. Sections reveal fibrosis and hemorrhage of the bowel wall with no macroscopic tumor invasion. On the left anterior surface, in the region of the trigone, is a 2.2 x 2.2 x 1.1 cm circumscribed, papillary mass with only superficial invasion into the mucosa. On the posterior surface is a 0.7 x 0.7 x 0.1 cm small red papillary focus. The remaining bladder mucosa is faintly bosselated and pink. Sectioning through the perivesical adipose tissue reveals 4 probable lymph nodes. Tumor and normal tissue are obtained for the (per protocol. Sections: K1 – right posterior bladder; K2 – right lateral; K3 – right

anterior; K4 – mid anterior; K5, K6 – papillary mass, left anterior; K7 – small papillary lesion, posterior; K8 – bladder dome; K9, K10 – right and left urethral margin and K11 – lymph nodes. A mapping diagram is utilized.

L. Received in formalin is a 2.5 x 1.7 x 0.9 cm aggregate of adipose tissue containing several 0.2 to 0.6 cm lymph nodes. All nodular tissue is submitted in L1.

Slides were microscopically examined by the pathologist.

Evaluation performed by [REDACTED]

Electronically signed [REDACTED]

Criteria	Yes	No
Diagnosis Discrepancy		/
Primary Tumor Site Discrepancy		/
HIPAA Discrepancy		/
Prior Malignancy History		/
Dual/Synchronous Primary		/
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	BTH	Date Reviewed: 8/2/13